NCCP 00327 · VERSION 5

Your treatment, explained

Progressive unresectable or metastatic well-differentiated pancreatic neuroendocrine tumour under NCCP 00327a. · NCCP 00327 v5 · Document revised 5 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Sunitinib 37.5 mg · pancreatic NET

Continuous daily capsules

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call urgently for severe diarrhoea or dehydration, bleeding, new swelling, much less urine, severe skin blistering or symptoms of low blood sugar. Chest pain, severe breathlessness, collapse, seizure or severe headache with vision changes needs emergency assessment.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your treatment and urgent-contact card is supplied separately and in the wallet-card section of the complete handbook.

Sources: nccp · smpc

Reference contents

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

How each drug works
How your treatment fits together
Your 28-day timeline
Baseline and cycle-specific checks
Whole treatment, scans and checks
Tablets, visits and practical planning
Your sunitinib treatment
Taking capsules and keeping safe
Urgent symptoms and treatment changes
Understanding your particular NET
Making sense of tests and results
Hormone symptoms and day-to-day changes
Your overall plan and choices
Eating, energy, practical help and support
Supportive medicines
Understanding the evidence
Questions to ask
Sources

UNDERSTAND YOUR TREATMENT

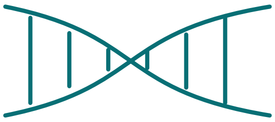
How your medicines work

Sunitinib is a daily targeted capsule used to help control a pancreatic NET that cannot be removed or has spread and is progressing. The team reviews control, side effects and your priorities.

TARGETED THERAPY CAPSULE

Sunitinib

Sutent



Blocks several growth and blood-vessel signals

Sunitinib blocks several signalling proteins involved in tumour growth and blood-vessel formation. This can slow a pancreatic NET. Effects on healthy blood vessels and other tissues help explain high blood pressure, bleeding, wound-healing problems, thyroid changes and sore hands or feet.

Growth / vessel signals → Receptors blocked → Growth may slow

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What to watch for

Keep blood-pressure and blood-test appointments; report bowel, skin, heart or bleeding symptoms early.

Sources: smpc

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

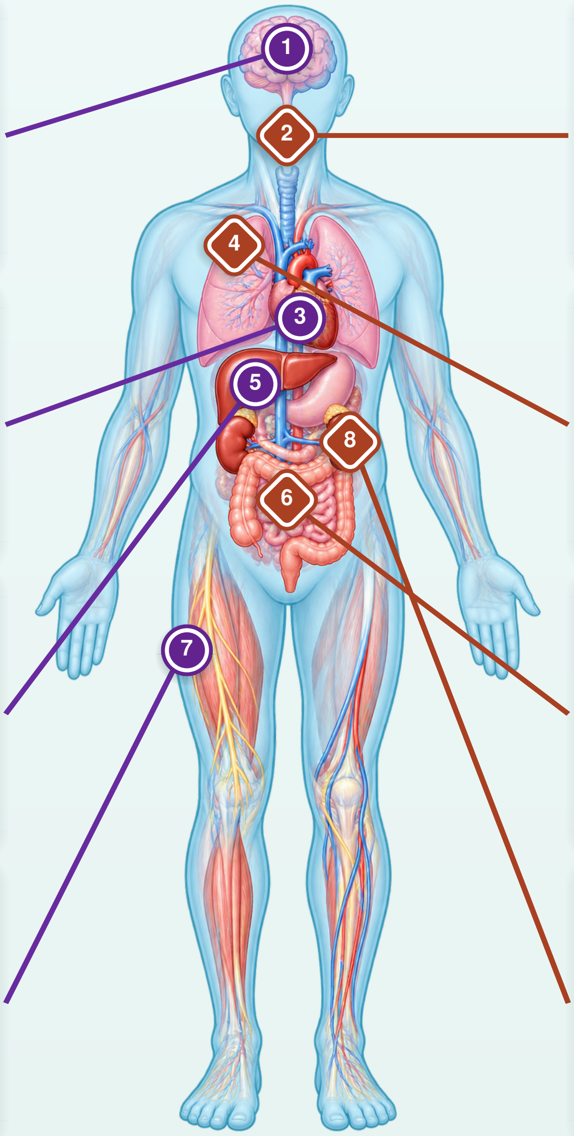
The anatomical side-effect map follows.

REGIMEN-SPECIFIC SIDE EFFECTS

Your anatomical side-effect map

A picture of side effects

Match each symptom box to its numbered body location. Tell your team about new symptoms.



1 Head / thyroid / energy

Sunitinib

Thyroid changes can contribute to tiredness. Severe headache, seizure, confusion or visual change needs emergency assessment.

2 Mouth / jaw

Sunitinib

Report mouth soreness, eating problems or jaw pain. Discuss dental work with your team before treatment or procedures.

3 Heart / blood pressure

Sunitinib

Blood pressure may rise without symptoms. Keep checks; new swelling or breathlessness needs advice, and chest pain needs emergency care.

4 Blood / bleeding

Sunitinib

Low counts can increase infection or bleeding risk. Report fever, sudden illness, unusual bruising or bleeding; heavy bleeding is an emergency.

5 Liver

Sunitinib

Liver tests may change before symptoms appear. Attend blood tests and report yellow skin or eyes, dark urine or severe abdominal pain.

6 Bowel / fluids

Sunitinib

Diarrhoea can cause dehydration. Follow the written plan and call for persistent symptoms, dizziness or inability to drink.

7 Hands / feet / skin

Sunitinib

Report painful redness, swelling or blisters early. Severe widespread blistering or peeling requires urgent help.

8 Kidneys / glucose

Sunitinib

Urine protein and kidney or glucose changes may need tests. Report reduced urine, swelling, shakiness, sweating or confusion.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

THE BIGGER TREATMENT PLAN

How your treatment fits together

28-DAY REVIEW

Continuous daily sunitinib

Daily Review

This is the 37.5 mg continuous pNET schedule, not a four-weeks-on/two-weeks-off schedule. Continue only as authorised by the team.

The written prescription and team review determine treatment; this timeline is a reference.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

Treatment safety, NET response and follow-up

Your team books and reviews each test. Response scans and hormone or tumour markers follow an individual plan; they are not automatically required at every medicine review.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00327 v5 · current Irish regimen · EMA product information and patient leaflet

THE WHOLE TREATMENT JOURNEY

Sunitinib 37.5 mg · pancreatic NET: treatment and checks

Progressive unresectable or metastatic well-differentiated pancreatic neuroendocrine tumour under NCCP 00327a.

Take the prescribed sunitinib dose once daily continuously, at the same time, consistently with or without food. The 28-day cycle is a review period, with no planned capsule-free week.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment / reviews	Between visits	Response review	After a pause or stop
Blood count / kidneys / liver NCCP 00327	Baseline tests	Before each 28-day cycle	Earlier tests if symptoms or dehydration	Review results before prescribing	Follow persistent abnormalities

Care / check	Before starting	Treatment / reviews	Between visits	Response review	After a pause or stop
Blood pressure / heart NCCP 00327	Baseline BP; cardiac assessment and echo/ MUGA if indicated	BP at every attendance; heart tests if indicated	Report chest pain, breathlessness or swelling	Agree management of BP or heart changes	Individual follow-up
Thyroid / urine / glucose NCCP 00327	Thyroid tests; baseline urine assessment; glucose review	Thyroid every 12 weeks; urine/glucose as clinically indicated	Report cold intolerance, marked fatigue or low-glucose symptoms	Review medicine effects and diabetes treatment	Follow unresolved changes
NET response NCCP 00327	Record symptoms, disease and personal goals	Consultant-directed imaging / markers	Track symptoms and adherence	Assess benefit and treatment burden	Plan next treatment or surveillance

What each check is for

1

Continuous daily sunitinib

28-day review

Sunitinib is a daily targeted capsule used to help control a pancreatic NET that cannot be removed or has spread and is progressing. The team reviews control, side effects and your priorities.

Ask / act: Confirm the current prescription, tests and support medicines.

Sources: NCCP 00327

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.37

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Take the prescribed sunitinib dose once daily continuously, at the same time, consistently with or without food. The 28-day cycle is a review period, with no planned capsule-free week.

Treatment timing: displayed days 1–28

Treatment or check	Days in this window	What happens
Sunitinib every day	Days 1–28	No planned rest week.
Symptoms and blood pressure	Days 1–28	Report changes promptly.
28-day review	Day 28	Counts, kidneys/liver and prescription review; BP at every attendance.

Continuous does not mean unchecked: Take capsules daily only as prescribed; reviews, test results or toxicity can lead to a pause or dose change.

After a hold: Record the last dose and reason. Your oncology team decides when and at what dose to restart.

No borrowed rest week: Other sunitinib schedules use breaks. This pancreatic NET regimen is continuous; follow its own calendar.

> **Why these bands stay broad:** Treatment timing is shown for orientation. Symptoms can develop on any day; no symptom band predicts an individual toxicity peak. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00327 v5 · current Irish regimen ↗

> **There are no guaranteed "safe days".** Follow your oncology unit's emergency instructions. Fever, shivering or suddenly feeling unwell needs urgent advice; do not wait for the next appointment. Call emergency services for collapse, severe breathing difficulty, chest pain or uncontrolled bleeding.

Complete timing and symptom notes

Day 1–7: Start with a clear prescription

Confirm the continuous dose, blood-pressure plan, skin care and other medicines. Record any existing bowel, mouth, skin or heart symptoms.

Day 8–28: Keep taking daily unless told to hold

Use the written symptom plan and report changes early. A new review window does not create a break. Attend review bloods; thyroid tests are due every 12 weeks.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Capsules and missed doses

The usual starting dose for this pancreatic NET schedule is 37.5 mg once daily. Follow your label, swallow whole with water and take at the same time each day consistently with or without food. If a dose is missed, do not take an additional dose; take the usual prescribed dose the next day.

STEP 2

Interactions

Avoid grapefruit and St John’s wort. Ask the pharmacist to review every medicine and supplement, including blood thinners, antibiotics and medicines affecting heart rhythm.

STEP 3

Surgery and dental work

Tell your team about planned operations, wounds or invasive dental work. A treatment interruption may be needed for major surgery; the oncology and surgical teams agree when to stop and restart. Ask about a dental assessment.

STEP 4

Storage and supply

Keep capsules in their original labelled packaging, away from children, and follow storage instructions. Return unused medicine to the pharmacy. Bring your current prescription and medicine list to review.

STEP 5

Pregnancy, feeding and fertility

Tell the team about pregnancy, planned pregnancy or breastfeeding before treatment. Ask for the contraception and fertility plan specific to sunitinib, including how long precautions continue after stopping.

STEP 6

Tests and visits

Baseline blood count, kidney/liver, thyroid and blood pressure; urine protein assessment and cardiac tests as indicated. Counts, kidney and liver tests before each cycle; BP at each attendance; thyroid every 12 weeks. Heart, urine and glucose monitoring are individualised. Scans and tumour markers follow the consultant plan.

Sources: NCCP 00327 v5 · current Irish regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Skin and mouth care

Use the unit’s gentle skin and mouth-care plan. Report hand-foot pain before it limits walking or daily tasks; the team may adjust treatment.

2 Sickness and diarrhoea

This oral regimen is minimal-to-low emetogenic. Routine premedication is not required. Agree rescue anti-sickness and antidiarrhoeal medicines and when to call.

3 Blood pressure and glucose

Follow the agreed BP plan. If you use diabetes medicines, ask how to monitor glucose and manage low readings; sunitinib may lower blood sugar.

4 Nutrition and fatigue

Report poor intake, weight loss or troublesome fatigue. Thyroid changes, anaemia, dehydration or other causes may need treatment.

Sources: nccp · smpc

TREATMENT REFERENCE · 01

Your sunitinib treatment

This guide is for the continuous pancreatic NET regimen.

Why sunitinib?

Sunitinib is a daily targeted capsule used to help control a pancreatic NET that cannot be removed or has spread and is progressing. The team reviews control, side effects and your priorities.

Sources: nccp · smpc

How it works

Sunitinib blocks several signalling proteins involved in tumour growth and blood-vessel formation. This can slow a pancreatic NET. Effects on healthy blood vessels and other tissues help explain high blood pressure, bleeding, wound-healing problems, thyroid changes and sore hands or feet.

Sources: nccp · smpc

What the trial can tell us

Sunitinib delayed progression compared with placebo in this population. The trial stopped early; early survival findings require caution and do not predict one person’s lifespan. The results apply to the pancreatic NET population and continuous dose studied, not every NET site or another sunitinib schedule.

Sources: trial

TREATMENT REFERENCE · 02

Taking capsules and keeping safe

Use your individual prescription and symptom plan.

Capsules and missed doses

The usual starting dose for this pancreatic NET schedule is 37.5 mg once daily. Follow your label, swallow whole with water and take at the same time each day consistently with or without food. If a dose is missed, do not take an additional dose; take the usual prescribed dose the next day.

Sources: nccp · smpc

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Tell your team about planned operations, wounds or invasive dental work. A treatment interruption may be needed for major surgery; the oncology and surgical teams agree when to stop and restart. Ask about a dental assessment.

Sources: nccp · smpc

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Baseline blood count, kidney/liver, thyroid and blood pressure; urine protein assessment and cardiac tests as indicated. Counts, kidney and liver tests before each cycle; BP at each attendance; thyroid every 12 weeks. Heart, urine and glucose monitoring are individualised. Scans and tumour markers follow the consultant plan.

Sources: nccp · smpc

Skin and mouth care

Use the unit’s gentle skin and mouth-care plan. Report hand-foot pain before it limits walking or daily tasks; the team may adjust treatment.

Sources: nccp · smpc

Sickness and diarrhoea

This oral regimen is minimal-to-low emetogenic. Routine premedication is not required. Agree rescue anti-sickness and antidiarrhoeal medicines and when to call.

Sources: nccp · smpc

Blood pressure and glucose

Follow the agreed BP plan. If you use diabetes medicines, ask how to monitor glucose and manage low readings; sunitinib may lower blood sugar.

Sources: nccp · smpc

Nutrition and fatigue

Report poor intake, weight loss or troublesome fatigue. Thyroid changes, anaemia, dehydration or other causes may need treatment.

Sources: nccp · smpc

TREATMENT REFERENCE · 03

Urgent symptoms and treatment changes

Call when a new problem develops.

When to seek help

Call urgently for severe diarrhoea or dehydration, bleeding, new swelling, much less urine, severe skin blistering or symptoms of low blood sugar. Chest pain, severe breathlessness, collapse, seizure or severe headache with vision changes needs emergency assessment.

Sources: nccp · smpc

Do not restart a held dose yourself

The team may pause, reduce or stop treatment for toxicity or test abnormalities. Record the reason and agreed restart plan; do not use another patient's dose or timetable.

Sources: nccp · smpc

Storage and supply

Keep capsules in their original labelled packaging, away from children, and follow storage instructions. Return unused medicine to the pharmacy. Bring your current prescription and medicine list to review.

Sources: nccp · smpc

Pregnancy, feeding and fertility

Tell the team about pregnancy, planned pregnancy or breastfeeding before treatment. Ask for the contraception and fertility plan specific to sunitinib, including how long precautions continue after stopping.

Sources: nccp · smpc

TREATMENT REFERENCE · 04

Understanding your particular NET

Several different features help the team choose treatment.

Where it started and where it is now

A neuroendocrine tumour (NET) starts in cells that receive signals and can release hormones. The original organ is the primary site. A pancreatic NET is different from the more common pancreatic adenocarcinoma. Stage describes how far the cancer has spread; the stage system depends on its original site. A tumour in the liver may be a NET that spread there from elsewhere.

Sources: nccn-patient

Differentiation, grade and Ki-67

Differentiation describes how the cells look under the microscope. Grade describes their growth activity. Ki-67 is a laboratory measure of the proportion of tumour cells showing a marker of cell growth and helps assess grade in relevant NETs. It is not the percentage of your body affected and is not a survival percentage. Ask for the exact pathology wording, grade and Ki-67 result, including the sample date.

Sources: nccn-patient

Grade is different from stage

A low-grade NET can have spread, and a high-grade tumour is not automatically stage 4. Well-differentiated grade 3 NET and poorly differentiated neuroendocrine carcinoma are different diagnoses with different treatment choices. These medicine guides must not be applied to another diagnosis simply because its name contains neuroendocrine.

Sources: nccn-patient

Functioning and nonfunctioning

A functioning NET causes a hormone-related syndrome. A nonfunctioning NET does not cause a recognisable excess-hormone syndrome, but it can still grow or cause symptoms by its size or location. Ask which description applies to you. A positive somatostatin receptor scan does not by itself mean the tumour is functioning.

Sources: nccn-patient · nci-pnet

Your diagnosis record

Keep the primary site, differentiation, grade, Ki-67 if reported, stage or sites of spread, hormone syndrome and receptor-scan result together in your passport. Ask the team to explain changes rather than trying to interpret one result in isolation.

Sources: nccn-patient

TREATMENT REFERENCE · 05

Making sense of tests and results

Safety tests, tumour tests and hormone tests answer different questions.

Blood tests for treatment safety

Blood counts measure infection-fighting cells, oxygen-carrying cells and platelets. Kidney and liver tests help the team assess how your body can handle treatment. The schedule for your medicine is shown separately. Attend these checks even when you feel well; a scan does not replace them.

Sources: nccn-patient · nccp

CT, MRI and receptor imaging

CT and MRI help show tumour size and where it is. A somatostatin receptor (SSTR) scan looks for receptor targets that certain treatments can attach to. An FDG-PET scan answers a different question about tumour activity and may be useful in selected cases. You will not necessarily need every scan. The specialist decides which test can change your care.

Sources: nccn-patient

Hormones and tumour markers are individual

The team may measure selected blood or urine markers when they answer a question about your NET or symptoms. These can include serotonin-related testing such as 5-HIAA for selected patients, or specific pancreatic hormones. Chromogranin A is not a universal test before every treatment. A changing marker alone does not prove cancer growth. Record why each test is being done, how to prepare and who will explain it.

Sources: nccn-patient · nci-net

Preparing for a test

Ask whether you need fasting, a timed urine collection, dietary preparation or a medicine review. Follow the laboratory's instructions for your particular test. Do not stop an acid-suppressing medicine, diabetes medicine or hormone-control treatment yourself. Tell the team about kidney problems, allergies and all medicines before a scan or blood test.

Sources: nccn-patient · nci-net

Heart and inherited-risk checks

A heart ultrasound may be needed for particular treatment risks or to assess heart-valve problems associated with a hormone syndrome. It is not required on the same timetable for every NET. Depending on the diagnosis, age and family history, the team may offer genetic counselling; some NETs occur with inherited conditions such as MEN1. A tumour gene test and an inherited-risk test are not interchangeable.

Sources: nccn-patient

Who does what, and when?

Use the passport to write each test, its purpose, planned date, booking service, result-review contact and next step. Your consultant sets response-scan and marker intervals. The postoperative schedules in general guides are not automatically the right schedule while you are taking these medicines.

Sources: nccn-patient · nccp

TREATMENT REFERENCE · 06

Hormone symptoms and day-to-day changes

A new symptom may come from the NET, treatment or another illness.

Build a picture of what is normal for you

Record your usual bowel pattern, flushing, pain, appetite, weight and energy before starting. Note what changes, how long it lasts, what you were taking and how it affects daily life. This helps the team distinguish a hormone problem from treatment toxicity without assuming that every symptom is cancer progression.

Sources: nccn-patient

Hormone-related symptoms

Depending on the tumour, excess hormones may cause flushing, frequent watery stools, wheezing, acid-related pain or blood-sugar disturbances. An insulin-producing tumour can cause low blood sugar. Not every NET causes these problems. Ask which symptoms matter for your diagnosis and whether you need a separate rescue plan.

Sources: nccn-patient

Hormone treatment is a separate part of the plan

Somatostatin analogue injections and other symptom treatments may continue alongside anticancer treatment when prescribed. They may have their own appointment dates. Never stop them simply because tablets are paused. PRRT requires a carefully coordinated interruption and restart plan, which the nuclear-medicine service must supply.

Sources: nccn-patient · nccp

When a change is urgent

Follow your oncology emergency plan for sudden or severe symptoms. Collapse, severe breathing difficulty, chest pain, confusion or uncontrolled bleeding needs emergency assessment. Severe diarrhoea, persistent vomiting, dehydration, fever or sudden illness needs urgent oncology advice. Do not wait for a scan, an online answer or the next clinic visit.

Sources: nccp

Procedures and hormone syndromes

Tell the surgical, dental, anaesthetic or imaging team if you have a hormone-producing NET or a previous hormone crisis. Ask your specialist whether special preparation is needed. Carry your current medicines and emergency plan; the wallet card supports this information but does not replace specialist instructions.

Sources: nccn-patient · nccp

TREATMENT REFERENCE · 07

Your overall plan and choices

Your priorities belong in the treatment plan.

What are we trying to achieve?

Ask whether the main aim is to slow growth, reduce tumour size, relieve hormone symptoms, make another treatment possible or protect day-to-day quality of life. The aim and likely benefit depend on your particular NET and previous treatment. A stable scan may represent useful control even without tumour shrinkage.

Sources: nccn-patient

How treatments fit together

Some people need surgery, hormone-control treatment, tablets, chemotherapy, PRRT, liver-directed treatment or external radiotherapy at different points. These are not all automatically part of your regimen. Ask the multidisciplinary team to draw your own sequence and name the clinician coordinating it.

Sources: nccn-patient

Choosing between options

Ask why this option suits your tumour site, grade, growth rate, symptoms, receptor status and other health conditions. Discuss alternatives, the option of observation when appropriate, supportive care, and any suitable clinical trial. A treatment listed in a US guide is not a guarantee of availability or funding in Ireland.

Sources: nccn-patient

Evidence is about groups

A trial can measure time before cancer worsens, tumour shrinkage or survival. These are different outcomes. A group result does not predict your own response or lifespan. Ask how closely the study patients match your diagnosis and what benefit would make the treatment worthwhile to you.

Sources: nccn-patient

Review points and shared decisions

Agree when you will review symptoms, tests and scans, and what would lead to continuing, delaying, changing or stopping treatment. Discuss work, caring duties, travel and the effects you most want to avoid. You can revisit your choices as your situation changes.

Sources: nccn-patient

TREATMENT REFERENCE · 08

Eating, energy, practical help and support

Supportive care can be used alongside treatment from the start.

Food, weight and digestion

Tell the team about weight loss, poor appetite, early fullness, persistent diarrhoea or greasy stools. Ask for a dietitian familiar with NETs and your surgery or medicines. Different causes need different support; do not assume every bowel symptom needs the same diet or medicine. Ask whether digestive-enzyme assessment or replacement is relevant to you.

Sources: nccn-patient · nci-pnet

Fluids and safe activity

Follow your individual fluid advice, especially with diarrhoea, kidney disease or heart problems. Balance manageable activity with rest. Tell the team if fatigue, dizziness, pain or breathlessness limits usual tasks; these symptoms can have treatable causes.

Sources: nccn-patient · nccp

Emotions, relationships and quality of life

Living with repeated tests and treatment can affect sleep, mood, relationships and confidence. Ask about psychological support, sexual health, fertility advice, symptom-control services and support for carers. Supportive or palliative care aims to improve comfort and daily life and can accompany active anticancer treatment.

Sources: nccn-patient

Practical help

Ask your nurse or social worker about transport, work, finances, home support, caring responsibilities and local NET support organisations. A general resource list may include services in other countries; your team can identify options available to you.

Sources: nccn-patient

A record that works without a phone

Keep the printed schedule, medicine list, tests, contact numbers and written emergency instructions together. The full handbook contains the information in sequence for offline reading. The QR code is an optional route to more information; it is not needed to call for help or follow your paper plan.

Sources: nccn-patient

Questions to take to the next visit

What is my exact diagnosis and grade? What is treatment trying to achieve? Which tests are mandatory for this medicine, and which depend on my NET? Who reviews results? What symptoms mean call now? What support would help me live as well as possible?

Sources: nccn-patient

Evidence for this continuous pancreatic NET schedule

The randomised trial compared daily sunitinib with placebo, with supportive care.

Sunitinib delayed progression compared with placebo in this population.

Sources: trial

PREPARE FOR THE CONVERSATION

Questions worth asking

Is my dose the continuous pancreatic NET schedule?

For this NCCP pancreatic NET regimen, the standard starting schedule is 37.5 mg once daily continuously, with 28-day reviews and no planned rest week. Your own prescribed dose may differ.

Who checks my BP and when are my 12-week thyroid tests?

Blood pressure is checked at each attendance and thyroid tests every 12 weeks, with extra tests if indicated. Ask who will book them and where to record readings.

What should I do about diarrhoea or painful hands and feet?

Report these symptoms early and follow the written rescue and hold plan. Painful hands or feet can require interruption even at the first episode; the team confirms dose changes and restarting.

Do my diabetes or blood-thinning medicines need extra monitoring?

Ask the pharmacist to check the full medicine list. Diabetes may require closer glucose checks and changes to diabetes treatment; bleeding risks and interactions need individual review.

What is the plan for dental work or surgery?

Tell the oncology team before arranging procedures. Surgery, wounds and dental work may need a coordinated interruption, with restart decided by the treating teams.

When will we review response and quality of life?

Ask for the planned symptom, scan and treatment-review dates. There is no fixed total number of cycles; continued treatment depends on benefit, tolerability and your priorities.

> **Try explaining the plan back:** Can you explain why this is continuous daily dosing, where to record blood pressure and when painful hands/feet or diarrhoea need a call?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00327 v5. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00327 v5 · current Irish regimen
- EMA product information and patient leaflet
- NCCN Guidelines for Patients: Neuroendocrine Tumors, 2026 · education reference
- NCI PDQ: gastrointestinal NETs · selected marker background
- NCI PDQ: pancreatic NETs · hormone and digestive background
- Raymond et al. phase III sunitinib pancreatic NET trial

SACTCheck · Developed by Dr Paul O’Brien · Sources checked 2026-11-05.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6919/327_V5_SUNitinib_37.5mg.pdf

https://www.ema.europa.eu/en/documents/product-information/sutent-epar-product-information_en.pdf

https://www.nccn.org/patients/guidelines/content/PDF/neuroendocrine-patient.pdf

https://www.cancer.gov/types/gi-neuroendocrine-tumors/hp/gi-neuroendocrine-treatment-pdq

https://www.cancer.gov/types/pancreatic/hp/pnet-treatment-pdq

https://pubmed.ncbi.nlm.nih.gov/21306237/